

UPMC Presbyterian

200 Lothrop St, Pittsburgh, PA 15213

DISCHARGE SUMMARY Document #: DIS-011

Patient & Admission

Name	Natasha Wood
Date of Birth	1946-05-06
Phone	878-555-5313
Address	Butler, PA, 16001
MRN	MRN-980841
Admit Date	2026-04-13
Discharge Date	2026-04-20
Attending	William Crawford, MD
Attending NPI	7468723433

Insurance

Primary Payer	Medicare
Primary Member #	9HUQ-09N-1699

Primary Discharge Diagnosis

Acute on chronic systolic heart failure

ICD-10: I50.22

Hospital Course

The patient presented to the emergency department with acute dyspnea and orthopnea, reporting a three-day history of progressive shortness of breath and lower extremity edema. Physical examination revealed elevated jugular venous pressure, bilateral crackles at the lung bases, and 2+ pitting edema to the knees. Initial laboratory studies demonstrated elevated BNP at 1,240 pg/mL and mild elevation in troponin at 0.04 ng/mL. Chest radiography confirmed pulmonary edema with bilateral infiltrates. Transthoracic echocardiography revealed a left ventricular ejection fraction of 28% with global hypokinesis, consistent with her known ischemic cardiomyopathy. The patient was admitted to the telemetry unit with a diagnosis of acute on chronic systolic heart failure, likely precipitated by medication non-adherence and dietary indiscretion.

During the hospital stay, the patient was treated with intravenous diuretics, achieving negative fluid balance of approximately 4.5 liters over the first four days. Furosemide was uptitrated to 80 mg twice daily, and lisinopril was increased to 20 mg daily as hemodynamics tolerated. Troponin remained stable without further elevation, and serial EKGs showed no acute ischemic changes. The patient's respiratory status improved significantly with diuresis, and she was transitioned to oral medications by hospital day five. Renal function remained stable throughout admission with creatinine ranging from 1.1 to 1.3 mg/dL. She was discharged on hospital day seven in stable condition with improved exercise tolerance and resolution of orthopnea.

At discharge, the patient was counseled extensively on sodium restriction, daily weights, and medication adherence. She was prescribed furosemide 80 mg daily, lisinopril 20 mg daily, carvedilol 12.5 mg twice daily, and spironolactone 25 mg daily. Close outpatient cardiology follow-up was arranged for one week post-discharge with repeat echocardiography planned in three months to reassess LV function and guide further optimization of her heart failure regimen.

Procedures Performed

IV diuresis with furosemide	—
------------------------------------	---

Medication Changes

Metoprolol tartrate	STOPPED
Spironolactone 25 mg daily	STARTED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	13 days
Urgency Tier	MEDIUM
Clinical Flags	EF<40%

Dictated and electronically signed by:
William Crawford, MD
NPI: 7468723433
UPMC Presbyterian